California Orthopaedic Association

## Sample Medical-Legal Report

*This document is presented as an example of the form and general content of an orthopaedic QME/AME report. The specific diagnoses, conclusions, and opinions expressed herein are those of the author and not necessarily those of the California Orthopaedic Association or every California Orthopedist.*

(Put on AME's Letterhead)

(Date of Evaluation)

Department of Industrial Relations
Division of Worker's Compensation
Disability Evaluation Unit
(Address)

| | |
|---|---|
| Re: | Tom Patient |
| SSN: | (Social Security Number) |
| D/I: | (Date of Injury) |
| C/N: | (Ins. Co. Claim #) |
| WCAB: | (WCAB Claim #) |
| Imp: | (Name of Employer) |
| D/Exam: | (Date of Exam) |
| D/Dict: | (Date Report Dictated) |

## Agreed Medical Evaluation – Orthopaedic Surgery

The above-captioned patient was seen in this office on January 27, 2005 for an orthopaedic evaluation with the undersigned acting in the capacity of an Agreed Medical Examiner.

This examination was assisted by the presence of a professional Spanish interpreter, (Name & Certificate #).

The patient is a 33-year-old, right-handed, Hispanic male, who worked in a factory that made kitchen cabinets. His job was to paint the cabinets. He states he would have to lift cabinets and he would have to lift up to 200 pounds with assistance. This was a full-time job. He started this job in June or July 2004 and had no concurrent employment.

The patient gives the following history. (In spite of the presence of an interpreter, it is very difficult to get a good history from the patient who changes his history.)

The patient reports that on November 30, 2004 he was lifting a cabinet with a co-worker and the cabinet started to fall and he held onto it and felt a pulling in his lower back. He completed the remaining five hours of his work shift. He says the pain was not very severe initially.

He remained at work for the next two to three days. The pain was getting worse and he reported it. He was sent to a chiropractor at the end of his work shift. He says he was only examined and given no treatment. Initially he states he was not taken off work but later, in trying to find out when he stopped working, he says he was told not to work. He has never returned to work since then.

He says that rather than getting treatment in the office of that doctor he was advised just to apply ice or heat.

At the second evaluation in that office, he was also given no treatment.

Following that, the company sent him to a medical doctor whose name he cannot recall. He says x-rays were taken. He was given medication and started on physical therapy with electrical stimulation and aquatherapy three times weekly. He had the treatment for about two to three months. When this treatment ended, he was still feeling the same. He says he was then released from care and told to go back to work.

The patient did not go back to work. He called in and was told he would be called back but he was never called back.

He then got an attorney. The attorney sent him to Dr. B. He was prescribed a back support, a TENS unit for home use, and medication. Later he was started on physical therapy, going three times weekly for electrical stimulation and acupuncture. Exercises were given but they bothered his back. He is still getting physical therapy and in spite of this prolonged treatment he still feels the same.

Additionally he says he was sent by Dr. B to Dr. F who gave him injections twice a week for the neck and back. (These sound almost like epidural injections, but getting epidural injections twice a week for quite some time does not sound reasonable.) He indicates he cannot estimate how many of these injections he had, but he says he did have a lot. He says with the injections he only got relief that lasted for three days.

He has had diagnostic studies that sounds like an EMG and MRI study, but he does not know the results of these studies.

## Present Complaints

The patient reports headaches. He says that since this injury he has to wear his glasses more frequently than previously. (Please note: a straining injury of the lower back should not have any effect on the need for use of glasses.)

He has neck pain. He indicates the neck pain first started approximately one and one half months after this injury. Initially it was only a very mild pain. He says he did tell the first doctor about the neck pain but the neck was not treated.

With reference to the neck he states there is a constant pain that is aggravated by looking up or down or lying on a pillow. The pain from the neck radiates into the lower back. The pain from the neck also radiates into both upper extremities to the fingers of both hands, more strongly in the left than the right. He says the pain going into his hands started when his neck pain got worse. He says the neck pain is stronger than the arm pain. Additionally there is tingling in both hands.

He reports low back pain that is a constant pain. The lower back pain is stronger than the neck pain. The lower back pain is aggravated by lying down, by standing and by sitting. (Please note: He did frequently change from standing to sitting while giving his history.)

He says that for relief of his back pain he takes medication and a change of position helps.

The lower back pain radiates to the toes of both feet, more strongly on the left than the right. The back pain is stronger than the leg pain. There is numbness and tingling in both legs. The back pain is aggravated by coughing and sneezing.

## Previous Injury History

The patient reports no history of a previous injury involving his neck or back, and no previous injury of any kind whatsoever. He reports no subsequent injuries.

He states this was his only lifetime injury.

## Personal History

The patient has a common-law living arrangement with one child. Tobacco: None. Alcohol: None. Medications: The patient is taking Tylenol, Ranitidine, Cardosoprol, and Naprosyn. Allergies: None.

## Past Medical History

The patient has had the usual childhood diseases. Serious Illness: The patient states he has an ulcer history. Surgeries: None. Hospitalizations: None.

## Physical Examination

The patient is a well-developed, well-nourished, Hispanic male. He comes to the evaluation utilizing a cane which he says he was given by Dr. B for the past month. He carries it in his left hand, but it was not used during the examination.

Generally throughout the examination the patient reports pain with almost every movement, and following performing range of motion studies he even says he feels "dizzy."

He gets on and off the examination table slowly. He walks very slowly. He says he cannot walk on his toes or heels because of pain. He does have a normal stance and habitus. He performs only 20% of a full squat with a complaint of back pain.

Vital Signs: Blood Pressure 110/90; pulse 80. Height 5 feet 8 inches. Weight 180 pounds. Right handed.

Examination of the Cervical and Dorsal Spine:

Inspection: The shoulders are level. The spine is straight. There is no deformity evident. There is a large hyperpigmented birthmark on the lower thoracic area.

Palpation: There is no muscle spasm. There is no localized tenderness. There is no increase in local warmth.

Motions (R/L): The patient flexes his chin to his chest. Extension is 60 degrees. Neck rotation is 60/40 degrees. Neck lateral bending is 30/30 degrees. (The patient complains of pain with all movement, and with rotation the pain is greater when he turns toward the left than the right.) Foraminal closure testing produces neck pain. Cervical compression produces marked low back pain but this is even prior to the application of any pressure whatsoever. Cervical traction similarly produces a complaint of pain in the neck area but before any pressure is applied that would produce traction on the neck. Shoulder depression testing is negative.

Motor Testing: Motor function in both upper extremities is intact.

Sensory Testing: The patient reports that he has relative diminution of sensation on the left side of his face (not including the forehead). There is also circumferential diminution of sensation in the left upper extremity that extends from the elbow to the fingertips. (This sensory distribution does not follow any anatomical neurological pattern.)

Reflexes (R/L): The biceps, triceps and brachioradialis reflexes are present and symmetrical.

Measurements (R/L = inches):

| | | |
|---|---|---|
| Circumference of upper arm | 11 | 10-3/4 |
| Circumference of forearm | 10-1/2 | 10-1/4 |

Jamar Dynamometer Grip Strength Testing (R/L): 100/85, 85/80, 90/75.

Examination of the Lumbosacral Spine:

Inspection: The pelvis is level. The spine is straight. No deformity is evident.

Palpation: There is no muscle spasm. There is no localized tenderness including examination by punch percussion. There is no increase in local warmth.

Motions (R/L): The patient makes no attempt to forward flex his spine because of complaint of pain. Extension is limited to 5 degrees because of pain. Lateral bending is 10/10 degrees. Trunk rotation is 30/30 degrees. (He indicates that lateral bending and rotation are also limited by pain.)

Supine leg raising by the patient at 40 degrees on the right and 30 degrees on the left with complaint of low back pain, but not sciatic pain. Fabere test is negative. Fajersztajn test is negative bilaterally. The reverse Fajersztajn test is positive on the right. (That is, at the limit of supine leg raising, dorsiflexion of the foot does not increase his pain, but plantar flexion of the right foot does increase his back pain. This is a maneuver which should relieve any stretch irritability and is a non-organic finding.)

The flip test (seated leg raising) is resisted by the patient at 5 degrees short of full extension of the knees, although he does not arch his back. The Cram test is negative. The patella shift test is negative.

Motor Testing: Motor function in both lower extremities is intact. (He complains of pain with resisted flexion and extension of the feet and toes while he lies supine.)

Sensory Testing: The patient reports relative diminution of sensation in the posteromedial left thigh and the medial aspect of the left calf.

Reflexes (R/L): Knee jerks 1+/1+; ankle jerks 0+/0+. There is no ankle clonus.

Measurements (R/L = Inches):

| | | | | |
|---|---|---|---|---|
| Leg length | | 37-1/2 | | 37-1/2 |
| Circumference of thigh | 18 | | 18 | |
| Circumference of calf | | 14 | | 14-1/4 |

## X-Ray Report

X-rays of the cervical, thoracic and lumbosacral spine, and pelvis were obtained in this office January 27, 2005 by Certified Radiology Technician C (Certificate #), and interpreted by the undersigned.

Cervical Spine (7 Views): There is no evidence of fracture, dislocation of any other bony injury. Disc and joint spaces are well maintained. There is no arthritic spurring. The neural formina are patent.

Thoracic Spine (3 Views): There is a slight right thoracic scoliosis curve. There is no evidence of fracture, dislocation, or other bony injury. Disc and joint spaces are well maintained. There is no arthritic spurring.

Lumbosacral Spine (5 Views): There are six lumbar-type vertebrae. (This is an anatomical variant.) There is no evidence of fracture, dislocation, or other bony injury. Disc and joints spaces are well maintained. There is no arthritic spurring. There is no spondylolysis. There is no spondylolisthesis.

Pelvis (1 View): There is no bony abnormality of the pelvis. The hip and sacroiliac joints are well maintained.

**Review of Records**

Some records have been submitted for review. These will be summarized in chronological sequence for clarity of review.

There is a Doctor's First Report of Work Injury prepared by Dr. B. who saw the patient on December 3, 2004. (I am not sure of the exact date because the report is poorly typed.) The date of injury is listed as November 30, 2004, and it was stated he had low back pain after trying to remove a heavy cabinet from a wall. The history is of neck and low back pain since November 30, 2004 after trying to remove a heavy cabinet from a wall that weighed about 100 pounds. It was noted he had help. The examination revealed slight kyphosis. He could reach to the floor but it was painful to recover. He had full rotation. Flexion to either side was limited to 40% of normal. It was stated he had an antalgic gait. He could heel and toe walk. X-rays of the lumbar spine showed no fracture and the diagnoses were lumbar strain and cervical strain. It was stated he was given medications and was to be checked by a doctor again on December 6, 2004. Handwritten on the form are some work restrictions that appear to be a 10 pound lifting restriction and he was to avoid repetitive bending, stooping or prolonged standing or sitting and he was to do no climbing. (It is somewhat hard to read this form.)

There is a typewritten work status report prepared by Dr. B on December 3, 2004 noting the diagnoses were a lumbar and cervical strain, and this was the first visit for the injury and it was stated he has had x-rays and was given medication. It was felt he could do modified work with a number of restrictions.

From that same facility there is a work status report prepared by Dr. W. dated December 7, 2004. The same diagnoses are listed and it was stated he was improving slowly and he could be at modified work with a number of restrictions.

Another report from Dr. W of December 13, 2004 again states he could be at modified work with the same restrictions. It was stated he was improving but slowly.

Next from that facility there is a report of an orthopaedic consultation prepared by Dr. Y. who examined the patient on December 30, 2004. In this report it was stated he had worked for the last 10 years for the company as a machine operator. (Please note: He told me he was hired in June or July 2004, a few months prior to the industrial injury.)

The history of injury of catching a falling cabinet is noted, with the development of an immediate pull and sharp pain in the low back. It was stated he took some Tylenol and felt better but the pain started increasing and he reported it on December 2 and was seen at the care station on December 3, 2004. He was given medication. It was stated he was having ongoing pain and was referred for further evaluation. He denied prior injuries to the neck or back. He was now complaining of severe pain, mostly in the low back, aggravated by prolonged sitting or standing. He could not arise from a sitting position on his own because of pain. Tylenol and other medications had helped only temporarily. The pain was not radiating beyond the buttocks. He had difficulty sleeping. There was some sharp pain and heaviness in the neck area that did not radiate into the arms but the neck pain was minor compared to the low back. He was currently not working.

Physical examination revealed he was in moderate distress with pain, and his gait was very slow and antalgic on the left, but normal reciprocating. He could toe and heel walk. Neck range of motion was completely normal. There was some tenderness of the upper trapezius, but no spasm. Lower back examination revealed decreased lordosis and tenderness and moderate to severe spasm. There was limited motion in all directions (but the motion was much greater than that demonstrated by the patient to the undersigned currently). It was noted leg raising was negative at 90 degrees bilaterally with negative stretch signs. Neurological examination as described revealed symmetrical reflexes and no loss of strength or sensation. It was noted x-rays of the lumbar spine were normal and the diagnosis was a severe lumbosacral strain and a mild cervical strain. It was noted that his findings were consistent with a strain and the examination showed no evidence of a radicular component. It was felt he should have conservative

treatment with medication and a course of physical therapy. He was released to modified work with a lot of restrictions.

A brief work status report from Dr. Y of December 30, 2004 notes that he was given Motrin and Parafon Forte and was to have a home interferential unit and be at modified work. He was also to start physical therapy three times weekly.

A physical therapy evaluation dated January 6, 2005 notes the patient was complaining of constant severe pain in the mid lumbosacral spine with tingling and numbness to the left lower extremity and throbbing pain in the neck without radiation. The pain appeared to have been aggravated since injury. He was to have a treatment program with a pool program, modalities and other treatments.

On January 20, 2005 a work status report from Dr. Y notes he was still being followed with the same diagnoses, something is mentioned about getting an MRI study but I cannot read the handwritten note clearly, and he was still to be at modified work.

There is a report of an MRI study of the cervical spine that was performed on February 3, 2005. There was a 2-3 mm disc protrusion at C6-7 with no neurological involvement. (Please note: An isolated disc protrusion at one level in the neck would not be related to a straining injury that involved the lower back.)

A follow-up report from Dr. Y is dated February 10, 2005 and notes the patient was complaining of increasing pain and there was now pain radiating down the legs and he wanted a back support. There was numbness and tingling. He was taking over-the-counter Tylenol because the other medicines had upset his stomach. It was stated therapy was only helping “somewhat.” He was not working. Neck range of motion was normal, although it was noted that with extension there was “exaggerated facies.” Lumbosacral motion revealed only a 10 degree limitation of flexion and extension, and extension was painful. Other motions are normal. The neurological examination as described was normal. MRI findings were noted. It was stated there was also an MRI of the lumbar spine (not available to the undersigned) that showed mild desiccation at L3-4 with a mild bulge that did not impinge on neural elements. The diagnoses of Dr. Y were “Cervical and lumbosacral strain. Subjective complaints much greater than any objective findings.”

Under the discussion it was noted that although he still had significant subjective complaints, they were not backed by objective findings. It was noted the MRI showed the spine was “structurally sound with no evidence of any danger to any of the neural elements or the spine itself.” It was explained to the patient that the best thing to do would be to exercise and he was released to regular duty.

The only other report available is a very brief note from I. Boyarsky, D.O. which is a disability status report and not a full medical report. It was stated the patient was seen on March 22, 2005 and he was temporarily disabled from that date through April 28, 2005, and that he needed treatment. No diagnosis is made and no examination findings are reported.

This concludes the review of the submitted records.

## Comment

After a review of the history as obtained directly from the patient and the records that have been submitted, my physical examination and the x-rays available, it is my opinion that the above-captioned patient has the following orthopaedic conditions:

1. History of cervical strain.
2. History of lumbar strain.

It has been more than six months since this patient's injury, and instead of showing any gradual improvement, his symptom complexity is expanding. I would concur with the final impression of Dr. Y that the best management program for this patient would be a progressive exercise program. I do not believe that intensive physical therapy treatments that are not reducing his pain are indicated, necessary or likely to be of any benefit.

I believe that for all practical purposes, it would be in the best interests of the patient and all parties concerned to consider his condition is at MMI at this time and settle his industrial injury claim.

The patient's condition has reached MMI subsequent to January 1, 2005, and will be rated according to the AMA Guides to Evaluation of Permanent Impairment, 5th Edition.

The patient's neck injury falls into the DRE cervical Category I.

It is noted there are no significant clinical findings. There is no muscle guarding or spasm. He has full range of motion, although he complains of pain at the extremes. He has some non-organic findings including the marked low back pain even prior to the application of any pressure with cervical compression. His sensory changes do not follow any organic neurological distribution. The motor function is intact, reflexes are normal and grip strengths are appropriate.

I see no evidence of permanent impairment and no objective signs. I recommend this patient do appropriate stretching and neck exercises before and after vigorous activity.

This patient's neck condition should be rated as 0% impairment of the whole person as per the AMA guides (pages 392).

With reference to his lower back condition, I would rate his impairment in the DRE category of DRE lumbar Category II.

It is noted his history and examination are consistent with a specific injury on a specific date. At the time of the examination he has marked restriction of motion; however, this appears to be more voluntary than objective impairment. He will not even attempt to perform any flexion. This is in contrast with his initial examination which shows excellent motion. His leg raising signs do not confirm any radicular finding and he has non-verifiable radicular complaints. There is no alteration of the structural integrity and no significant radiculopathy.

DRE Lumbar Category II rates to 5% to 8% impairment of the whole person, and I believe that the non-objective factors which predominate would put this patient in a 5% category of whole person impairment.

Using the combined value chart of a 5% impairment plus a 0% impairment results in a 5% whole person impairment for this individual.

At the current time, I do not believe that any ongoing medical care is indicated or necessary but the patient should be encouraged to pursue a regular home exercise program. He should be reassured regarding the absence of any significant pathology in his back or neck.

Giving the patient every benefit of the doubt, I believe that at most he should have a permanent work restriction in the category that would preclude very heavy lifting. Whether or not he could return to his usual and customary work would best be determined by reviewing an analysis of the physical demands of his usual and customary work. If an analysis can be obtained, I would be happy to review it and provide a supplemental report.

In terms of apportionment based upon causation, the information available indicated this patient has only had one injury in his lifetime. There is no evidence of any other factors contributing to his disability. Therefore, I would apportion all of his current condition to industrial causation, namely the lifting and straining injury that occurred at work on November 30, 2004.

The patient was seen at this time for orthopaedic evaluation purposes only. If there are any questions regarding these opinions, please do not hesitate to call upon me.

Please note, obtaining a history directly from the patient (with the assistance of an interpreter, if needed), reviewing and summarizing all records (if any), performing the entire physical examination and all measurements, reviewing x-rays (if any), and the preparation and dictation of this entire report were entirely and solely performed personally by the undersigned. The only exceptions were the measuring of vital signs and the transcription of my personally dictated findings onto an office work sheet which were done by (office manager), who has been my office manager and back office assistant for 20 years and has been instructed and monitored in these activities by me. The time spent in connection with this evaluation was in compliance with any available guidelines.

I declare under penalty of perjury that there has not been a violation of Labor Code Section 139.3 that the contents of the report are true and correct to the best of my knowledge, and any statements concerning any bill for services are true and correct to the best of my knowledge.

I declare under penalty of perjury that the information contained in this report and its attachments, if any, is true and correct to the best of my knowledge and belief, except as to the information that I have indicated I have received from others. As to that information, I declare under penalty of perjury that the information accurately describes the information provided to me and, except as noted herein, that I believe it to be true.

Date_____ in the County of_____

Sincerely yours,

Joe AME
Diplomat of American Board of Orthopaedic Surgery

CC: Adjustor
Defense Attorney
Applicant Attorney

Fortunes Rocks MD Consultants

William F. Boucher, MD

725 Main Street · South Portland, ME 04106
Phone: 207.879.0404 · Fax: 207.879.0606
www.fortunesrocksmd.com

# Independent Medical Evaluation Report

| | |
|---|---|
| **Examinee:** | Tom Sample |
| **File/Claim Number:** | 000123456 |
| **Date of Examination:** | September 18, 2004 |
| **Examining Physician**: | William F. Boucher, MD |
| **Examination Location:** | Vermont |
| **Date of Birth:** | January 1, 1964 |
| **Date of Injury:** | December 18, 1985 |
| **Client Organization:** | The Best Insurance Company |
| **Referral Source:** | Miss Adjuster |

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## Introduction

This 40-year old, right-handed man was referred for an independent medical evaluation (IME) by the above client. The independent medical examination process was explained to the examinee, and he understands that no patient/treating physician relationship was established. Mr. Sample was advised that the information provided will not be confidential and a report will be sent to the requesting client. The client's questions were described to the examinee.

Mr. Sample was cooperative. History was provided by the examinee who was a good historian. The information he provided was consistent with the medical records provided. Accompanying Mr. Sample was his wife who waited outside.

A questionnaire and pain inventories were completed by the examinee. To ensure accuracy, the clinical history was dictated in his presence.

Mr. Sample reported no difficulties occurring during the examination.

The client provided the following clinical records: The Medical Center and One Memorial Hospital.

These records were carefully reviewed, and listed for the examinee. No records prior to December 18, 1985 or subsequent to June 14, 2004 were available to review. These records will be purged from this file. The report, questionnaires, pain inventories and other material specific to this evaluation will be retained. The following records were not available at the time of this evaluation: None.

## History

**Preexisting Status**

Mr. Sample denies any previous problems or injuries, including any other work- or liability-related injuries. Mr. Sample also denies having any difficulties similar to those he is now experiencing prior to the date of injury.

**Injury**

Mr. Sample reports that on December 18, 1985, he fell on the ice landing on his left hip.

At that time the difficulties were fracture of the left hip.

**Clinical History**

The examinee was transported to the Hospital Emergency Department, then transported at his request to One Medical Center. Examination at that time revealed shortening and external rotation of the left lower extremity, pain with internal and external hip rotation. X-ray revealed a left intra trochanteric fracture. The examinee was seen by Dr. Kinko, who on December 19, 1985, performed open reduction and internal fixation of the fracture. The examinee did have postoperative complications including pulmonary edema, which resolved promptly with treatment.

The examinee was then discharged with gradually increasing weight bearing. By February 2, 2004, the examinee was ambulating with a cane. He was referred to physical therapy. Initial evaluation revealed mildly decreased left hip motion.

By mid March 2004, Dr. Kinko noted essentially normal gait with painless hip motion. Hip motion was not measured at that time. The examinee's left leg was noted to be one-half inch short and the patient was given a lift.

By mid June 2004, x-rays revealed healing of the fracture with some ongoing impaction. Gait was normal. The examinee was released from Dr. Kinko's care at that time. The examinee has required no further treatment for his left hip condition.

**Diagnostic Studies:**

| Study | Date | Result |
|---|---|---|
| X-Ray Left Hip | 6/18/03 | Fixation hardware in good position; fracture healed with slight impaction |

**Current Status**

The examinee's chief concern is whether it is a "permanent injury."

He reports difficulties with pain which is primarily located in the left hip. The pain is described as minimal. The pain is worsened by lying on the left side and prolonged walking. The pain is reported as intermittent.

On a scale from 0 (no pain) to 10 (excruciating pain), the examinee reports the pain now is a 2. During the past month the pain averaged 2, with a high of 4 and a low of 1.

**Functional Status**

Examinee states he is able to lift a gallon of milk; able to lift a heavy bag of groceries; able to lift a pail of water. Examinee states that he can sit for up to two hours, stand for up to two hours and walk for up to two miles.

**Occupational History**

At the time of the injury, he had been employed by The Fort at #4 and had been working there for two years. He was working full-time as a museum administrator. According to the description provided by the examinee, the job involved operation of a museum and staff. He has a college education.

In terms of current work status, he is currently not working. He last worked on January 12, 2004.

**Social History**

The examinee lives in Charleston, NH. The activities of a typical day include "seeking new job." He denies performing any other work activities or vigorous recreational pursuits.

The examinee does not smoke or drink.

**Past Medical History**

| | |
|---|---|
| Medical: | Chron's disease |
| Surgery: | Bypass surgery |
| Medications: | See list |
| Allergies: | None known |

**Review Of Systems**

Noncontributory

**Family History**

Noncontributory

## PHYSICAL EXAMINATION

### Observations

The examinee is a well-developed, well-nourished male. He appears healthy. Examination of the hands reveals mild callus. No assistive devices were used. Height was 5 feet, 7 inches and weight 160 pounds as reported by the examinee.

### Behavioral Observations

The examinee was pleasant, cooperative and attentive. Affect was normal. During the visit, he appeared comfortable. He sat continuously for up to 20 minutes. There was no significant pain behavior. Non-physiologic findings were not present.

### Structural Examination

In the standing neutral position, cervical, thoracic and lumbar curves were well-maintained. The shoulders were symmetric. The right PSIS was elevated (as the examinee was not wearing his heel lift). Gait was normal with no antalgia. There was no external rotation of the left foot.

### Lower Extremity Examination

Inspection revealed a well-healed surgical scar over the lateral left hip. Hip motion as measured with a goniometer was symmetrically decreased in external rotation, but otherwise normal.

| MOTION | NORMAL | LEFT | RIGHT |
|---|---|---|---|
| Forward flexion | 100 | 120 | 120 |
| Extension | 0 | 0 | 0 |
| Abduction | 25 | 35 | 35 |
| Adduction | 15 | 30 | 30 |
| Internal Rotation | 20 | 35 | 35 |
| External Rotation | 30 | 15 | 15 |

The examinee had no pain with hip motion. Resisted hip abduction and external rotation was pain free. There was no tenderness of hip structures. There was no left thigh atrophy. However, leg length measured from the lateral pelvic rim to the lateral tibial plateau revealed a 2.5 cm deficit on the left.

## PAIN STATUS INVENTORIES

### Pain Drawing

The examinee completed a pain drawing (attached), using symbols to describe sensations. This drawing did reveal findings suggestive of symptom magnification.

### Pain Disability Index

The Pain Disability Index uses rating scales to measure the extent of perceived disability in seven areas of life. The results are as follows:

| Area | Perceived Disability |
|---|---|
| Family/home responsibilities | 20% |
| Recreation | 20% |
| Social Activity | 0% |
| Occupation | 10% |
| Sexual activity | 40% |
| Self-care | 0% |
| Life-support activities | 0% |

The total score is 9 out of a possible 70, for a total index of 13%. This indicates a mild degree of perceived disability.

### CES-D

The Center for Epidemiologic Studies Depressed Mood Scale was administered. The examinee scored 10, which is not consistent with a depressed mood.

## CONCLUSIONS

### Diagnoses

1. Left hip fracture, intra trochanteric
   1.1 Status post open reduction and internal fixation
2. Status post fracture left ankle
3. Coronary Artery Disease
4. Diabetes
6. History depression

### Discussion

The examinee's injury of December 18, 1985 was a left hip fracture which did not involve the hip joint. He underwent open reduction and internal fixation (without joint replacement) and has done very well. He does generally use a heel lift due to left leg shortening, but is currently walking two miles per day with no difficulty.

### Causation

Based upon the available information, to a reasonable degree of medical certainty, there is a causal relationship between the examinee's current left hip complaints and the reported injury. The examinee's more diffuse low back symptoms are of uncertain origin, given the degree of symptom magnification present. More likely than not, these symptoms are due to pre-existent (mild) facet arthropathy, obesity and symptom magnification.

### Work Capacity

This examinee has at least a sedentary work capacity as defined in the *Dictionary of Occupational Titles*, U.S. Department of Labor. At this time, the examinee can lift perhaps 10 pounds occasionally and five pounds frequently. He assumes that he is unable to sit, but I can find no objective reason why that should be the case. In truth, it is likely that he could sit and stand for reasonable periods of time, requiring frequent position changes. He could walk for perhaps 15 minutes per hour. He can bend and twist occasionally, but should involve spotting. He can kneel and crawl occasionally and climb stairs minimally. He should avoid ladders. He can push and pull with perhaps 20 pounds effort. He would have no other restrictions regarding upper extremity use. He should avoid driving a motor vehicle or operating machinery due to her multiple sedating medications.

Prior to his July 2004 hip surgery, he would have had essentially the same work restrictions, except that his duration for sitting, standing and walking may have been somewhat greater.

### Appropriateness of Care

The client has asked that I specifically address the issue of appropriateness of medical care. Based on the specifics of this care, it is my professional opinion that care has not been entirely consistent with the usual standards of care for this problem. In particular, the use of Avinza is inappropriate given the lack of objective reason for the examinee's pain complaints and the ineffectiveness of the medication. Furthermore, use of Neurontin has been inappropriate given the lack of evidence of neuropathic pain. Finally, the failure to address the examinee's probable significant depression has been inappropriate.

**Prognosis**

The overall prognosis is good.

**Maximum Medical Improvement**

The examinee has achieved maximum medical improvement. MMI is defined as the date after which further recovery and restoration of function can no longer be anticipated, based upon a reasonable degree of medical probability.

**Permanent Impairment Evaluation**

Permanent impairment evaluation was performed in accordance with the AMA *Guides to the Evaluation of Permanent Impairment*, Fifth Edition. The examinee warrants impairment on the basis of a 2.5 cm limb length discrepancy. According to Table 17-4 (page 528), this imparts 7% lower extremity impairment, which converts to 3% whole person impairment.

**Recommendations**

**Diagnostic/Consultation**

No further diagnostic testing or consultation is indicated.

**Therapeutic**

The examinee will continue to require a heel lift of perhaps 1.5 cm. No other treatment will be necessary for his left hip condition.

## Qualifications

I am a founding member of the American Board of Independent Medical Examiners, having served on the Selection Committee, and am certified by ABIME. I have performed over 2000 Independent Medical Examinations and Permanent Impairment Ratings in the past 13 years. I have spoken to national groups on work fitness and disability issues, and have built a national reputation for my expertise in these issues. I have been a contributor to publications regarding disability issues and permanent impairment. I am board certified in Occupational Medicine, and am a Fellow of the American College of Occupational and Environmental Medicine and the American College of Preventative Medicine. I am past Chairman and current secretary of ACOEM's Work Fitness and Disability Evaluation Section. I maintain a clinical practice as Medical Director of WorkWell at Southern Maine Medical Center.

The above analysis is based upon the available information at this time, including the history given by the examinee, the medical records and tests provided, the results of pain status inventories, and the physical findings. It is assumed that the information provided to me is correct. If more information becomes available at a later date, an additional report may be requested. Such information may or may not change the opinions rendered in this evaluation.

The examiner's opinions are based upon reasonable medical certainty and are impartial. Medicine is

both an art and a science, and although an individual may appear to be fit for work activity, there is no guarantee that the person will not be re-injured or suffer additional injury. If applicable, employers should follow the processes established in the Americans with Disabilities Act, Title I. The opinions on work capacity are to facilitate job placement, and do not necessarily reflect an in depth direct threat analysis. Comments on appropriateness of care are professional opinions based upon the specifics of the case, and should not be generalized, nor necessarily be considered supportive or critical of, the involved providers or disciplines.

Any medical recommendations offered are provided as guidance, and not as medical orders. The opinions expressed do not constitute a recommendation that specific claims, or administrative action be made or enforced.

Sincerely,



William Boucher, M.D., C.I.M.E.

WB/ls

To: David J. Freeland
2171 Chile Pepper Blvd.
San Chalopos, CA 93001

Re: Sample, Manuel

Date of Injury/Onset: May 1, 2007

Dear Mr. Freeland:

On May 7, 2007, Mr. Manuel Sample presented himself for an initial examination and evaluation of his complaints arising from a motor vehicle accident that he was involved in on May 1, 2007.

# CASE SUMMARY NARRATIVE

## ACCIDENT DESCRIPTION:

The time was 7:00pm. Mr. Sample stated that he was the driver in a SUV which was stopped at a light at approximately 0 m.p.h. According to the patient, the other vehicle involved was travelling at approximately 25-30 m.p.h. He stated that the other vehicle struck his vehicle in the rear end.

Mr. Sample also reported that, at the time of the accident, the road conditions were clean and dry. In addition, he stated the damage to his SUV was mild due to the fact that the other car hit his trailer hitch. Damage to the other vehicle was moderate. He also stated that he did not see the accident coming, and therefore was not braced for the impact. Also, he was wearing his seat belt and had his shoulder harness on. On impact, neither the driver's or the front passenger's air bag deployed.

His SUV was equipped with headrests. He also noted that he had his head facing straight forward at the moment of impact. On impact the patient's body did not strike the inside of his vehicle. He stated that he did not lose consciousness during the accident. Mr. Sample reports that he and his wife went to Seton Medical Center the following day. They were given a prescription for Ibuprofen, an analgesic, and Soma, a muscle relaxer and released. On release he was given instructions to rest and ice his injuries. The patient did not have x-rays taken following his injury.

## INITIAL COMPLAINTS:

Immediately following the accident, the patient's main complaints included neck pain and stiffness, pain in the low back, fatigue, and pain in the upper back. Following the accident Mr. Sample drove himself home.

## CURRENT COMPLAINTS:

Mr. Sample's current signs and symptoms were assessed today. His first symptom is sharp, aching and spastic pain in the neck bilaterally. He reported that the pain radiates into both shoulders. It occurs between three fourths and all of the time when he is awake, and causes *serious* diminution in his capacity to carry out daily activities. It is aggravated by bending forward, bending backward, bending to the left, bending to the right, twisting to the left, twisting to the right, coughing, sneezing, straining and by lifting.

Mr. Sample's second stated symptom is sharp, aching and spastic pain in the low back bilaterally. It also radiates into his buttocks. It occurs between three fourths and all of the time when he is awake, and causes *serious* diminution in his capacity to carry out daily activities. It is aggravated by bending forward, bending backward, bending to the left, bending to the right, twisting to the left, twisting to the right, coughing, sneezing, straining, standing and by lifting.

He stated his third symptom is dull, aching and spastic pain in the upper back bilaterally. It occurs between three fourths and all of the time he is awake, and is tolerated but it does cause *some* diminution in his capacity to carry out daily activities. It is aggravated by bending forward, bending backward, bending to the left, bending to the right, coughing, sneezing, straining, standing and by lifting.

## HISTORY:

Mr. Sample indicated that he had not experienced prior symptoms similar to his current complaints, and was symptom free at the time of the aforementioned accident/onset of May 1, 2007.

I have determined that Mr. Sample’s history has not contributed to his present condition.

### Prior Treatment Information:

The patient reported that prior to his first visit to this office, he was treated at Seton Medical Center. His first visit there was on May 2, 2007. During his visits to that office, Mr. Sample received examination and pain meds, which he reported had little, if any, benefit. The patient is no longer receiving treatments at that office.

## ACTIVITIES OF DAILY LIVING ASSESSMENT:

Based on an assessment of Mr. Sample’s history, along with his subjective complaints, objective findings, and other test results, it is evident from a standpoint of medical certainty, that his current condition did result from the type of injury/onset described in this report. He reported suffering varying degrees of losses of functional capacity with the following activities:

With regard to *Self Care and Personal Hygiene*, Mr. Sample stated: making his bed can be performed, despite significant pain, but only if he has help; bathing, showering, washing his hair, drying his hair, combing his hair, washing his face, putting on his shirt, putting on his shoes, tying his shoes, putting on his pants, taking out the trash and going to the toilet can be managed by himself, despite marked pain; brushing his teeth, eating and cleaning dishes can be done without much difficulty, despite some pain.

With regard to *Physical Activity*, Mr. Sample stated: standing, standing for long periods, walking, stooping, squatting, sitting continuously, kneeling, bending forward, bending backward, bending to the left, bending to the right, walking for long periods, twisting to the left, twisting to the right, leaning forward, leaning backwards, leaning to the left, leaning to the right and kneeling for long periods can be managed alone, despite marked pain; sitting, reclining and reaching can be done, but not without some difficulty because of the resulting pain.

Regarding *Functional Activities*, Mr. Sample stated: lifting weights off the floor can be performed, despite significant pain, but only if he has help; carrying large objects, lifting weights off of a table, pushing things while standing and pulling things while standing can be managed by himself, despite marked pain; carrying small objects, climbing stairs and climbing up any type of incline can be done without much difficulty, despite some pain.

Regarding *Travel*, Mr. Sample stated: driving for long periods of time can be managed by himself, despite marked pain; driving a motor vehicle and riding as a passenger in a motor vehicle can be done without much difficulty, despite some pain.

## GENERAL PHYSICAL EXAMINATION:

Mr. Sample is a right-handed 37 year-old mentally alert and cooperative male.

**Date of Birth:** October 1, 1969.

His superficial appearance suggested he was in distress. **Minor's Sign** was present. This sign is present when the patient, in arising from a chair, leans forward, jackknifing the thighs and the dorsolumbar spine so that the head is over the feet. Using the hands on the thighs or the arms of the chair, the patient pushes the body to an upright position, thus sparing lower limb effort. The presence of this sign is usually indicative of sciatica and or low back pain.

**Gait:** His walk revealed a mild antalgic gait.

**Stature:** Well developed.

## RANGE OF MOTION STUDIES:

The following joint range of motion calculations and analyses were performed to determine Mr. Sample's present condition with regard to joint motion.

| **Cervical Spine:** | **Angle** | **Analysis** |
|---|---|---|
| Flexion | 65 degrees | Moderate restriction:norm is This test brought on both pain and spasms. | 75 degrees. |
| Extension | 45 degrees | Mild-moderate restriction: The above caused pain and spasms. | norm is 55 degrees. |
| Left Lateral Flexion | 30 degrees | Mild-moderate restriction: The above caused pain and spasms. | norm is 40 degrees, |
| Right Lateral Flexion | 30 degrees | Mild-moderate restriction: The above caused pain and spasms. | norm is 40 degrees, |
| Left Rotation | 65 degrees | Moderate restriction: The above caused pain and spasms. | norm is 80 degrees, |
| Right Rotation | 65 degrees | Moderate restriction: The above caused pain and spasms. | norm is 80 degrees, |

| **Thoracolumbar Spine:** | **Angle** | **Analysis** |
|---|---|---|
| Flexion | 90 degrees | No restriction: This test brought on both pain and spasms. | norm is 90 degrees. |
| Extension | 20 degrees | Moderate restriction: This test brought on both pain and spasms. | norm is 30 degrees. |
| Left Lateral Flexion | 25 degrees | Mild restriction: The above caused pain and spasms. | norm is 30 degrees, |
| Right Lateral Flexion | 25 degrees | Mild restriction: norm is The above caused pain and spasms. | 30 degrees, |
| Left Rotation | 30 degrees | No restriction: norm is 30 The above caused pain and spasms. | degrees, |
| Right Rotation | 30 degrees | No restriction: norm is 30 The above caused pain and spasms. | degrees, |

## ORTHOPEDIC EVALUATION:

**Cervical Lesion Tests:**
**The O'Donoghue Manuuver Test** was positive bilaterally. A positive with this test indicates muscular strain and/or ligamentous sprain. Mr. Sample exhibits a Type II cervical acceleration-deceleration injury with associated mild ligamentous sprain, muscle strain and spasm. **The Shoulder Depression Test** was positive bilaterally for localized neck pain, which indicates adhesions of the soft tissues. Production or aggravation of radicular pain indicates adhesions of the dural sleeves, spinal nerve roots, or adjacent structures of the joint capsule of the shoulder.
**The Cervical Compression Test** was positive for localized pain. Localized pain indicates possible foraminal encroachment. Radicular pain indicates pressure on the nerve root.

**Lumbar Lesion Tests:**
**Kemp's Test** was positive for local pain which suggests facet irritation, or a strain/sprain.
**The Belt Test** was positive for a lumbar lesion.
**The Straight Leg Raise Test** was positive and suggests sciatica from lumbosacral or sacroiliac lesions, subluxation syndrome, disc lesions, spondylolisthesis, adhesions, or IVF occlusion.
**Fajersztajn's or The Well Leg Raise Test** is a Straight Leg Raise done on the unaffected side. If pain is elicited on the affected side the test is positive and a strong indicator of a possible herniated disc.This test was positive.
**The Double Leg Raise Test** was positive. This test is specific and highly accurate for lumbosacral joint lesions.
**Nachlas' Test** was positive bilaterally. This test is considered positive when the patient experiences pain in the lumbosacral region, which was the case with Mrs. Gomez, or the sacroiliac region, or in both regions.
**Braggard's Sign** was positive and indicates possible disc herniation, sciatic neuritis, spinal cord tumors, or spinal nerve irritation.
**Ely's Sign** was positive. With the patient prone, the heel of the affected leg is approximated to the opposite buttock. After flexion of the knee, the thigh can be hyperextended. With irritation of the iliopsoas or it sheath, it will be impossible to extend the thigh. Pain in the anterior thigh is a positive finding and indicates inflammation of the lumbar nerve roots, especially the upper lumbar.

**Sacroiliac Lesion Tests:**
**Hibb's Test** was positive bilaterally. This test is performed with the patient in a prone position. The examiner, while stabilizing the pelvis on the side nearest to him, flexes the opposite knee to a right angle. From this position, the examiner slowly laterally pushes the leg causing strong internal rotation of the femoral head. The test is done bilaterally. Pelvic pain reveals a positive test.
**The Iliac Compression Test** was positive bilaterally. This test is performed with the patient lying on the side on a padded table. The examiner with both hands over the superior innominate bone applies strong downward pressure. When this maneuver

elicits pain from the pelvis, the test is considered positive, and indicates a sacroiliac lesion or a sprain of the posterior sacroiliac ligaments.
**Nachlas' Test** was positive bilaterally. This test is considered positive when the patient experiences pain in the lumbosacral region, the sacroiliac region, or in both regions.
**Yeoman's Test** with the patient prone, pressure is applied to the SI joint. The knee if flexed and the heel is brought to the same side buttocks. The hip is extended by lifting the knee off the table. This applies pressure to the anterior sacroiliac joint. If pain is increased in the sacroiliac area this indicates a sacroiliac injury.

**Sciatic Nerve Lesion Tests:**
**Braggard's Sign** was positive and indicates possible disc herniation, sciatic neuritis, spinal cord tumors, or spinal nerve irritation.

**Intervertebral Disc Syndromes:**
**The Bowstring Sign** was present bilaterally. This test is done with the patient supine. The examiner performs straight leg raising until the patient experiences some discomfort. At this level the examiner flexes the knee slightly and rests the foot on his or her shoulder until any pain subsides. The examiner then applies pressure to the hamstrings. If this doesn't produce pain, the examiner moves the thumbs over the popliteal fossa and applies pressure over the popliteal. If pain is reproduced in the leg or in the back, this sign is considered to be present and indicates nerve root compression.
**Fajersztajn's or The Well Leg Raise Test** is a Straight Leg Raise done on the unaffected side. If pain is elicited on the affected side the test is positive and a strong indicator of a possible herniated disc.This test was positive.
**Kemp's Test** was positive for local pain which suggests facet irritation, or a strain/sprain.

**Postural Evaluation:**
A mild anterior gravitation of the head was noted. An mild elevation of the shoulder on the left side was noted by visual inspection. When comparing the left and right sides of the ilium, a slightly higher crest on the left side was noted. Additionally, a mild increased lumbar lordosis was noted.

## ASSESSMENT/DIAGNOSIS:
It is expected that Mr. Sample will experience favorable results from his treatments.

Acute, moderate, constant, sprain/strain of the cervical spine with associated cervicalgia. Acute, moderate, constant sprain/strain of the lumbar spine with associated low back pain. Acute, slight, constant sprain/strain of the thoracic spine with associated thoracic pain. Subluxations of the cervical, thoracic, and lumbar spine and pelvis.

**ICD-9 Coding:** 847.0, 847.2, 847.1, 724.2, 723.1, 739.1, 739.3, 739.2, 724.1

## PROGNOSIS:

At this time, Mr. Sample's prognosis is good. His case is somewhat complicated, but continued improvement is expected, despite permanent residuals being a possibility.

## TREATMENT PLAN:

**Chiropractic Manipulative Treatment:** Chiropractic Diversified and Drop Table full-spine adjusting.

**Physical Therapy:** Microcurrent for tissue healing and pain control. Ice or heat as needed to reduce pain and inflammation. Therapeutic stretching and strengthening exercises to restore functional biomechanics.

**Nutritional support:** Nutritional supplements consisting of Omega-3 Fatty Acids, Glucosamine and Chondroitin Sulfates, and MSM to reduce pain and inflammation and to facilitate the healing processes.

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## FUTURE CARE PLAN:

**Present Care Phase:** Currently, we have the patient in a relief phase of care. This includes: Chiropractic adjustments, microcurrent, ice packs, and nutritional support.

**Future Treatment Plan:** Mr. Sample's future care plan includes home exercises, physical therapy, massage therapy, trigger point therapy, chiropractic adjustments, resistive strengthening exercises, and nutritional support.

**Goals of Treatment Plan:** The preceding treatment plan has the goal of decreasing pain, decreasing swelling and inflammation, decreasing spasms, increasing range of motion, increasing the ability to perform normal activities of daily living, increasing strength, returning the patient to his pre-accident status, increasing function, retarding degeneration, correcting muscle imbalance, increasing flexibility, achieving maximum chiropractic improvement, reducing frequency and severity of probable exacerbations and improving alignment.

## CLOSING COMMENTS:

Mr. Sample's complaints are consistent with the type and severity of accident he was involved in.

## PRIMARY TREATING PHYSICIAN:

**I declare that this report is true and correct to the best of my knowledge.**

**Signature:**

**Executed at:** ***Second the Motion Chiropractic*** **Date:** 05/15/07
**Name:** Buzz Lightyear, D.C. **Cal. Lic.#** DC70007
**Address:** 31001 Broadway Boulevard **Phone:(**605**)**383-3838
Los Ponchos, CA 90303